

SC05 REDUCE RESPIRATORY PARTICLE EXPOSURE | O (MAX: 1 PT)

Intent: Implement strategies to reduce human contact with respiratory particles.

Summary: This feature requires projects to implement design and policy strategies to minimize some instances of contact with contaminated respiratory particles.

Issue: Many viral diseases, including COVID-19²⁶² and influenza,²⁶³ are spread by oral or respiratory emissions of liquid particles emitted by an infected person when they cough, sneeze or even exhale.¹⁸² Factors that may affect exposure include the size distribution of the respiratory particles,²⁶³ humidity,^{264,265} air flow^{265,266} and air treatment.^{265,266} While the relative influence of these factors is variable, direct exposure to particles shed by an infected individual may increase a person's odds of acquiring certain diseases.^{262,267}

Solutions: Implementing design and policy strategies aimed at reducing exposure to some particles shed by infected individuals, like establishing physical distancing among people^{268,269} or providing barriers to prevent respiratory particles,²⁷⁰ may slow the spread of pathogens.²⁷¹

Part 1 Part 1 (Max: 1 Pt)

For All Spaces except Dwelling Units:

The following requirements are implemented during periods when higher incidence of respiratory disease is likely:

- a. At least one of the following distancing strategies:
 1. Queuing marks to increase distance between people while waiting in line (e.g., in elevator lobbies, at check-out counters) and while using moving sidewalks and escalators, as applicable.
 2. Screens, protective furnishings or other engineering controls to reduce particle exchange at security check-ins, reception areas, check-out counters and other places with frequent interaction between occupants and a stationary worker.
 3. Self-service systems to control ingress or egress to the project (e.g., at reception desks or checkout counters).
- b. At least one of the following circulation strategies:
 1. One-way hallways and corridors.
 2. Separate entry and exit doors at pedestrian building entrances.
 3. Separate entry and exit for restrooms except single-user bathrooms.

Note: Interiors projects may count base building elevators, entries and exits towards feature requirements, even if outside of the project boundary.

The following requirements are implemented during periods when higher incidence of respiratory disease is likely:

- a. All of the following in any shared spaces (e.g., meeting rooms, workspaces, communal kitchens):
 1. Strategies to increase distance among occupants.
 2. Expectations and requirements for usage of face coverings or personal protective equipment.
 3. Clearly communicated rules for occupancy to reduce respiratory particle exposure and rationale

for their use.

- b. The project or organization implements at least one of the follow communication strategies to educate occupants about the practices implemented by the project to reduce respiratory particle exposure:
 - 1. Monthly communication (e.g., email, webcast) to all regular occupants.
 - 2. Prominent signage (physical or digital) at all building entrances and in shared spaces.

Note:

This feature is a beta strategy and has an additional documentation requirement (beta feature feedback form). The feedback form supports IWBI in developing new features that are effective and applicable to projects around the world.